



FOOD FOR THOUGHT

Ryan Fernando



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Looking the part, losing the edge

As weight-loss injections gain popularity, athletes risk sacrificing muscle, resilience and performance in **pursuit** of a leaner physique.

A report recently claimed some Indian cricketers, including IPL players, are using weight-loss injections to look leaner on camera. The sourcing was anonymous. I cannot verify it and will not pretend otherwise, and nobody should be treated as guilty because a trend story exists.

But I am not dismissing the question either, because I get asked about it almost every week now. Not by patients with diabetes but by young fit people who do not need the drug at all, and increasingly by people who work in sport.

The timing is not a coincidence. Semaglutide, the molecule behind Ozempic and Wegovy, went off patent in India this year; more than 40 companies lined up generics and prices collapsed. Tirzepatide, sold here as Mounjaro, became India's largest-selling drug brand by monthly value last October, seven months after launch. When a drug turns cheap and fashionable within the same 12 months, sport has a problem.

GLP-1 is a hormone your gut releases after you eat. It tells the pancreas to release insulin, slows stomach emptying, and signals fullness to the brain. The drugs copy that signal and hold it for a week at a time, so you eat less because you want less. For Type 2 diabetes or obesity, that is a real clinical advance, and I have no argument with a doctor prescribing it for a cricketer with metabolic disease. My argument is with using it to chase a photograph.

Start with the rules. These drugs are not banned, are not on the 2026 WADA Prohibited List, and need no therapeutic use exemption. Markers of both molecules do sit on WADA's 2026 Monitoring Programme, in and

out of competition. Monitoring carries no sanction, but it is how WADA builds a case, and a decision is expected in a year or two. **Legal today is not necessarily legal for the rest of a career. A player buying an unlabelled vial through a grey channel is gambling with strict liability.**

The part that matters more: what the weight is made of.

In the body-composition substudy of STEP 1 on semaglutide, roughly 60 per cent of the loss was fat and 40 per cent lean tissue. In the SURMOUNT-1 substudy on tirzepatide, the split was closer to 75 and 25. A quarter to two-fifths of every kilogram lost comes out of muscle.

Here is the caveat nobody quotes. Those participants were living with obesity, with fat to spare. A 22-year-old fast bowler at 12 per cent body fat does not, and with little fat left to mobilise, the ratio is unlikely to be kinder to him.

The training that protects that muscle also seems to fall away. A study presented at ENDO 2026 in June, the first large look at wearable